CLINICAL SURGERY CASE PRESENTATION

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Patient’s particulars

Name- Mrs. ABC
Age- 65 Years
Occupation- home maker
Address- Madikeri
Educational status- higher secondary
Socio economic status- lower middle class
Date of admission- 1/3/2020
Date of examination-2/3/2020

CHIEF COMPLAINTS

Yellowish discoloration of eyes and dark colored urine since one and half months.
Pain abdomen since one month
Nausea and vomiting since one month

HISTOTRY OF PRESENT ILLNESS

Patient was apparently well one and half month back, then she developed yellowish discoloration of eyes and dark colored urine, which was insidious in onset, gradually progressive, not associated with waxing and waning of jaundice. It was associated with clay colored stool and generalized itching.

Patient also gives history of pain in the right upper quadrant since one month, which was dull aching type, mild to moderate in intensity, not associated with food intake, non radiating, non referred, no aggravating factor relieved on taking oral analgesic.

There is history of on and off episode of nausea and vomiting, vomiting is non projectile, small quantity, gastric digested food, non blood or bile tinged, non foul smelling, not associated with food intake.
History of dyspepsia, lose of appetite and lose of weight.

- No history of hematemesis, melena,
- No history of altered bowel habits and mass per abdomen.
- No history of evening raise of temperature, night sweats.
- No history of passage of worms in stool.
- No history blood transfusion, viral hepatitis, intravenous drug abuse or hepatotoxic drug intake.
- No history of hemolytic disorder or typhoid fever.
- No history of bleeding from orifices or purpura.
- No history of oliguria

For present complaint she had taken oral drug treatment from the local hospital and symptoms didn't subside, so now she is referred to our hospital.

Past history

Patient is diagnosed to have diabetes 6 months back and is on oral medication.
No history of TB, or abdominal surgery
No history of biliary colic or jaundice in the past.

Family history- No history suggestive of any malignancy.(pancreas, gallbladder, stomach, colon)

Personal history

Diet is mixed, appetite is reduced, sleep is undisturbed, regular bowel and bladder habits.
No history of any substance abuse
She had attained her menopause 10 years back.

Summary

65 year old female, presented with yellowish discoloration of eyes and urine since one and half month, dull aching pain in the right upper quadrant since one month, intermittent non bilious, non projectile vomiting since one month, with history of clay colored stool and itching, dyspepsia, loss of weight and lose of appetite.

General examination

Patient is conscious, cooperative well oriented to time , place , and person. She is moderately built and poorly nourished.(BMI- 19.3)

Vitals

- Patient is afebrile (98.9 F*) at the time of examination.
- Pulse- 68/min regular rhythm, good volume, no radio-radial and no radio femoral delay all peripheral pulse are felt.
- Respiration- 20/minute, thoraco abdominal
- BP- 140/80mmhg left arm, supine position.
- JVP is not raised

- Icterus is present.
- No pallor, cyanosis, clubbing, edema, generalized lymphadenopathy
- Scratch marks over the legs and forearm
- No signs of liver cell failure

Local examination

Per abdominal examination

Examined in supine position, exposure from nipple to mid thigh

Inspection

- Flat abdomen, umbilicus is in center and inverted
- corresponding quadrant moves equally with respiration.
- No scars, visible veins, visible gastric peristalsis, visible pulsations.
- All hernia orifices are intact
- No fullness or swelling in the left supraclavicular fossa.

Palpation

No local raise of temperature

Tenderness is present in right hypochondrium on deep palpation.

4x4 cm globular mass is palpable in right hypochondrium, all borders except superior border is well made out. Finger cannot be insinuated below the costal margin. Which has smooth surface and firm in consistency, moves with respiration, side to side mobility is present, skin over the swelling is normal.

Lower border of Liver is palpable 2cm below the right costal margin.

Spleen is not palpable.

Percussion

mass has impaired note on percussion

Liver span is 17cm from 5th intercostal space in the midclavicular line till 2cm bellow the right costal margin.

No signs free fluid in the abdomen.

Auscultation

Normal bowel sounds heard

There is no venous hums

Per rectal and per vaginal examination- because of lack of privacy not able to perform.

Other system examination

Cardiovascular system

Apex beat is present in 5th intercostal space half an inch medial to midclavicular line

S1, S2 heard, no added sound

Respiratory system

Bilateral air entry is equal, normal vesicular breaths sound

No added sounds

Central nervous system

No focal neurological deficit

Diagnosis-obstructive jaundice involving the distal end of common bile duct, most likely of malignant pathology, with possibility being

Differential diagnosis

carcinoma head of pancreas

Periampullary carcinoma

Cholangiocarcinoma